

Modifiers

A list of the most frequently used CPT (Current Procedural Terminology) modifiers, HCPCS (Healthcare Common Procedure Coding System) modifiers has been compiled for your reference.

Modifiers provide the means by which the reporting provider can indicate a service or procedure has been altered by some specific circumstance but has not changed in its definition or code.

Modifiers may be used to indicate that:

- A service or procedure has both a professional and technical component
- A service or procedure was performed by more than one physician
- A service or procedure has been increased or reduced
- Only part of a service was performed
- An additional service was performed
- A bilateral procedure was performed more than once
- Unusual events occurred

CPT MODIFIERS (Used in Medicare Part B)

- 22 Unusual procedural service - Surgeries for which services performed are significantly greater than usually required, may be billed with the "22" modifier added to the CPT code. Include a concise statement about how the service differs from the usual.
Supportive documentation, e.g., operative reports, pathology reports, etc., must be submitted with the claim. Note: Documentation requirement applies to New Jersey and New York
- 23 Unusual Anesthesia.
- 24 Unrelated Evaluation & Management service by the same physician during a postoperative period.
- 25 Significant, separately identifiable E&M service by the same physician on the same day of the procedure or other therapeutic service which has (0-10 day global period). A separate diagnosis is not needed. This modifier is used on the E &M service
- 26 Professional Component – Certain procedures are a combination of a physician component may be identified by adding the modifier 26 to the usual procedure number. All diagnostic testing with a technical and professional component done in an outpatient or inpatient setting must reflect the 26 modifier. The fiscal intermediary (Part A Medicare) will reimburse the facility for the technical component.
- 50 Bilateral procedure – Bilateral services are procedures performed on both sides of the

body during the same operative session or on the same day. Medicare will approve 150 percent of the fee schedule amount for those services.

- 51 Multiple Procedures – Internal use only by Carrier.
- 52 Reduced Services - Use modifier 52 (reduced service) to indicate a service or procedure is partially reduced or eliminated at the physician's election. **When you report modifier 52, include office records, test results, operative notes, or hospital records to substantiate the reason for reporting a reduced service. If this information is not included, your claim may be denied. Note: Documentation requirement applies only to New Jersey.**
- 53 Discontinued Procedure - Under certain circumstances, the physician may elect to terminate a surgical or diagnostic procedure. Due to extenuating circumstances, or those that threaten the well being of the patient, it may be necessary to indicate that a surgical or diagnostic procedure was started but discontinued. **Documentation must be submitted with the claim. Note: Documentation requirement applies only to New Jersey.**

One of the most common examples of modifier 53 (this is an exception to the rule) is when an incomplete colonoscopy is performed. Add modifier 53 to CPT code 45378. No documentation is required.

- 54 Surgical care only - When one physician performs a surgical procedure and another physician provides preoperative and/or postoperative management, the surgical service should be identified by adding modifier 54 to the usual procedure code.
- 55 Postoperative management only. When one physician performs the postoperative management and another physician has performed the surgical procedure.
- 57 Initial Decision for surgery (90-day global period). This modifier is used on E&M service, the day before or the day of surgery to exempt it from the global surgery package.
- 58 Staged or related procedure or service by the same physician during the postoperative period. If a less extensive procedure fails, and a more extensive procedure is required, the second procedure is payable separately. Modifier 58 must be reported with the second procedure.
- 59 Distinct procedural service - The physician may need to indicate that a procedure or service was distinct or separate from other services performed on the same day. This may represent a different session or patient encounter, different procedure or surgery, different site, separate lesion, or separate injury. However, when another already established modifier is appropriate, it should be used rather than modifier 59.
- 62 Two surgeons (co-surgery) - Under certain circumstances, the skills of two surgeons (usually with different skills) may be required in the management of a specific surgical procedure. Adding modifier 62 to the procedure code used by each surgeon should identify the separate. Services. **Documentation for the medical necessity for**

two surgeons is required. Note: Documentation requirement applies only to New Jersey.

- 66 Surgical team - Under some circumstances, highly complex procedures, requiring the accompanying services of several physicians, often of different specialties, plus other highly skilled, specially trained personnel, and various types of complex equipment, are carried out under the surgical team concept. **Documentation establishing that a surgical team was medically necessary is required. Note: Documentation requirement applies only to New Jersey**
- 76 Repeat procedure by same physician: Indicate the reason or the different times for the repeat procedure in item 19 of the CMS 1500 Form or the electronic equivalent,
- 77 Repeat procedure by another physician. Indicate the reason or the different times for the repeat procedure in item 19 of the CMS 1500 Form or the electronic equivalent.
- 78 Return to the operating room for a related procedure during the postoperative period. The physician may need to indicate that another procedure was performed during the postoperative period of the initial procedure. When this subsequent procedure is related to the first, and requires the use of the operating room, it should be reported by adding modifier 78 to the related procedure.
- 79 Unrelated procedure or service by the same physician during the postoperative period. The physician may need to indicate that the performance of a procedure or service during the postoperative period was unrelated to the original procedure.
- 80 Assistant surgeon. Add modifier 80 to the usual procedure in a non-teaching setting to identify surgical assistant services
- 82 Assistant surgeon when qualified resident surgeon not available in a teaching setting
- 90 Reference (Outside) Laboratory - When laboratory procedures are performed by a party other than the treating or reporting physician, the procedure may be identified by adding the modifier 90 to the usual procedure number. For the Medicare program, this modifier is used by Independent Clinical Laboratories when referring tests to a Reference Laboratory for analysis.
- 91 Repeat clinical diagnostic lab tests performed on same day to obtain subsequent reportable test value(s). This modifier is used to report a separate specimen(s) taken at a separate encounter.
- 99 Multiple modifiers - When more than two modifiers are needed use the 99 modifier. Subsequent modifiers need to be in Item 19 of the CMS 1500 claim form or in the narrative of an electronic claim.

HCPCS MODIFIERS

- AA Anesthesia services personally furnished by an anesthesiologist

AD	Medical supervision by physician: more than four concurrent anesthesia services
AH	Services provided by a Clinical Psychologist (Note: This applies only to New York)
AJ	Services provided by a Clinical Social Worker (Note: This applies only to New York)
AS	Physician assistant, nurse practitioner, or clinical nurse specialist service for assistant at surgery
CB	Services ordered by a dialysis facility physician as part of the ESRD beneficiary's dialysis benefit, is not part of the composite rate, and is separately reimbursable.
CC	Procedure code change (the carrier uses the CC when the procedure code submitted was changed either for administrative reasons or because an incorrect code was filed)\
EJ	Subsequent claim (for a defined course of therapy e.g., Erthropoietin (EPO))
E1	Upper left, eyelid
E2	Lower left, eyelid
E3	Upper right, eyelid
E4	Lower right, eyelid
FA	Left hand, thumb
F1	Left hand, second digit
F2	Left hand, third digit
F3	Left hand, fourth digit
F4	Left hand, fifth digit
F5	Right hand, thumb
F6	Right hand, second digit
F7	Right hand, third digit
F8	Right hand, fourth digit
F9	Right hand, fifth digit
GA	Advanced Beneficiary Notification on file
GC	This service has been performed in part by a resident under the direction of a teaching physician
GE	This service has been performed by a resident without the presence of a teaching

physician under the primary care exception

- GG Performance and payment of screening mammogram and diagnostic mammogram on the same patient, same day. (Effective for dates of service on or after 01/01/2002)
- GH Diagnostic mammogram converted from screening mammogram on same day. (Effective for dates of service on or after 01/01/2002)
- GJ "OPT OUT" physician or practitioner emergency or urgent service
- GM Multiple patients on one ambulance trip
- GN Service delivered under an outpatient speech-language pathology plan of care
- GO Service delivered under an outpatient occupational therapy plan of care
- GP Service delivered under an outpatient physical therapy plan of care
- GQ Via asynchronous telecommunications system
- GT Via interactive audio and video telecommunication system
- GV Attending physician not employed or paid under arrangement by the patient's hospice provider. (Effective for dates of service on or after 01/01/2002)
- GW Service not related to the hospice patient's terminal condition. (Effective for dates of service on or after 01/01/2002)
- GY Item or service statutorily excluded or does not meet the definition of any Medicare benefit
- GZ Item or service expected to be denied as not reasonable and necessary and Advanced Beneficiary Notification has not been signed.
- KD Infusion drugs furnished through implanted Durable Medical Equipment (DME) - (Effective January 1, 2004)
- KX Specific required documentation on file
- KZ New coverage not implemented by Managed Care.
- LC Left circumflex coronary artery
- LD Left anterior descending coronary artery
- LR Laboratory round trip
- LT Left side (use to identify procedures performed on the LEFT side of the body)
- QA FDA investigational device exemption
- QB Physician providing service in a rural HPSA

- QC Single channel monitoring (*recording device for holter monitoring*)
- QD Recording and storage in solid state memory by a digital recorder (*digital recording/storage for holter monitoring*)
- QJ Services/items provided to a prisoner or patient in State or local custody. However the state or local government, as applicable, meets the requirements in 42 CFR 411.4
- QK Medical direction of two, three or four concurrent anesthesia procedures involving qualified individuals
- QL Patient pronounced dead after ambulance called
- QP Documentation is on file showing that the laboratory test(s) was ordered individually or ordered as a CPT-recognized panel other than automated profile codes
- QS Monitored anesthesia care service
- QT Recording and storage on tape by an analog tape recorder
- QU Physician providing services in an urban HPSA
- QV Item or service provided as routine care in a Medicare qualifying clinical trial
- QW CLIA waived test
- QX CRNA service - with medical direction by a physician
- QY Anesthesiologist medically directs one CRNA
- QZ CRNA service - without medical direction by a physician
- Q3 Live kidney donor surgery and related services
- Q5 Service furnished by a substitute physician under a reciprocal billing arrangement
- Q6 Service furnished by a locum tenens physician
- Q7 One class "A" finding
Class "A" finding: Non-dramatic amputation of foot or integral skeletal portion thereof.
- Q8 Two class "B" findings
Class "B" findings: Absent posterior tibial pulse; Advance tropic changes (hair growth, nail changes, pigmentary changes, or skin texture - three required); absent dorsalis pedis pulse.
- Q9 One class "B" and two class "C" findings
Class "C" findings: Claudication; Temperature changes, edema, paresthesias; burning.

RC	Right coronary artery
RT	Right side (use to identify procedures performed on the RIGHT side of the body)
SG	Ambulatory Surgical Center (ASC) facility charges. This modifier is only used by the ASC for identifying the facility charge. It should not be reported by the physician when reporting his/her professional service rendered in an ASC.
TA	Left foot, great toe
T1	Left foot, second toe
T2	Left foot, third toe
T3	Left foot, fourth toe
T4	Left foot, fifth toe
T5	Right foot, great toe
T6	Right foot, second toe
T7	Right foot, third toe
T8	Right foot, fourth toe
T9	Right foot, fifth toe
TC	Technical component. Under certain circumstances, a charge may be made for the technical component of a diagnostic test only. Under those circumstances the technical component charge is identified by adding modifier TC to the usual procedure number.
UN	Transportation of portable x-rays, two patients served - (Effective January 1, 2004)
UP	Transportation of portable x-rays, three patients served - (Effective January 1, 2004)
UQ	Transportation of portable x-rays, four patients served - (Effective January 1, 2004)
UR	Transportation of portable x-rays, five patients served - (Effective January 1, 2004)
US	Transportation of portable x-rays, six patients or more served - (Effective January 1, 2004)
ZP	No purchased services. Note: This applies only to New York Has been deleted as of 1/1/03

AMBULANCE ORIGIN AND DISTINATION MODIFIERS

The following values **must be** used in combinations of two in order to form a two-position modifier. The modifier must indicate both origin and destination. A modifier must be entered for every trip.

Example: Modifier RH would be used for ambulance trip from the Residence to Hospital

The first position alphabetic value = origin of service.

The second position alphabetic value = destination of service

- D** Diagnostic or therapeutic site other than "P" (Physician's Office) or "H" (Hospital)
- E** Nursing Home, residential, domiciliary, custodial facility (other than a Skilled Nursing Facility - SNF)
- G** Hospital-based dialysis facility (hospital or non-hospital related)
- H** Hospital
- I** Site of transfer (e.g., airport or helicopter pad) between types of ambulance vehicles
- J** Non-hospital based dialysis facility
- N** Skilled nursing facility (SNF)
- P** Physician's office (includes HMO non-hospital facility, clinic, etc.)
- R** Residence
- S** Scene of accident or acute event
- X** (Destination code only) Intermediate stop at physician's office on the way to the hospital (include HMO non-hospital facility, clinic, etc.)